

Atezolizumab and bevacizumab

Atezolizumab and bevacizumab is a combination of 2 different cancer drugs. This treatment is used to treat primary liver cancer.

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What is atezolizumab and bevacizumab?

These drugs can be given together to treat a type of primary liver cancer called hepatocellular cancer (HCC).

It is best to read this information with our general information about [immunotherapy drugs](#) and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and the possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How these drugs work

Atezolizumab is an immunotherapy drug called a [checkpoint inhibitor](#).

[Bevacizumab](#) belongs to a group of targeted therapy drugs known as monoclonal antibodies. It is also an [angiogenesis inhibitor](#) which makes it difficult for the cancer tumour to develop the blood vessels it needs to get a blood supply.

How atezolizumab and bevacizumab is given

You usually have atezolizumab and bevacizumab at a day unit or clinic as an outpatient. You may have it with other cancer drugs.

During a course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During your course of treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your treatment.

Both these drugs can be given as a drip into a vein (intravenous infusion). Atezolizumab can also be given as an injection under the skin (subcutaneous).

After you have the drugs your cancer team may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

Injection under the skin

If you have atezolizumab as an injection under the skin the nurse will give it slowly over about 7 minutes. You usually have it into the thigh. The nurse will change the exact place each time you have treatment. Sometimes the area around the injection may become:

- painful
- swollen
- red, if you have white skin
- darker, if you have black or brown skin.

If this happens, tell your doctor, nurse or pharmacist. Painkillers may help.

Drip into a vein

Your first treatment of atezolizumab takes about 1 hour. If you do not have any problems, you can have the following treatments over 30 minutes.

Bevacizumab is usually given over 90 minutes. If you do not have any problems, you have the next treatment over 60 minutes. After this, you usually have it over 30 minutes.

Your nurse will give you these drugs through 1 of the following:

- a [cannula](#) – a short, thin tube the nurse puts into a vein in the arm or hand
- a [central line](#) – a fine tube that goes under the skin of the chest and into a vein close by
- a [PICC line](#) – a fine tube that is put into a vein in the arm and goes up into a vein in your chest
- an [implantable port](#) (portacath) – a disc that is put under the skin on the chest or arm and goes into a vein in your chest.

Your course of treatment

You have this treatment session once every 3 weeks with a rest period in between. The treatment session and the rest period make up 1 cycle of your treatment. Your cancer team can explain the number of cycles you may need. You may have a scan after the first 3 cycles to assess how the treatment is working.

Your doctor, nurse or pharmacist will discuss your treatment plan with you.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Because atezolizumab is an immunotherapy drug it can make the immune system too active. This can cause side effects that may be serious if not treated quickly. There is a section on immune system side effects below.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Delayed side effects

Atezolizumab may also cause delayed side effects. These can start weeks, months, or sometimes more than a year after treatment ends.

If you have symptoms or side effects that start during treatment or after treatment ends, contact the hospital straight away on the 24-hour number. Side effects can become serious if they are not treated.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Your patient alert card

They will also give you a card with information about your treatment. It is important to keep the card with you during treatment and after treatment ends. You should show it to any doctor or healthcare professional caring for you.

Side effects while treatment is being given

Some people have side effects while they are having a cancer drug or shortly after they have it.

Allergic reaction

Some people have an allergic reaction while having this treatment. The first infusion is the most likely to cause a reaction, so it is usually given more slowly than later treatments. Before treatment, you will have medicines to help prevent or reduce any reaction.

Signs of a reaction can include:

- feeling hot or flushed
- shivering
- itching
- a skin rash
- feeling dizzy or sick
- a headache
- feeling breathless or wheezy
- swelling of your face or mouth
- pain in your back, tummy or chest.

Your nurse will check you for signs of a reaction during your infusion. If you feel unwell or have any of these signs, tell them straight away. If you do have a reaction, they can treat it quickly.

Sometimes a reaction happens a few hours after treatment. If you develop any of these signs or feel unwell after you get home, contact the hospital straight away on the 24-hour number.

Immune system side effects

Atezolizumab can make the immune system too active. This can cause the following side effects. Some of them are rare. But it is important to know about them. This is why we mention these before the more common side effects.

Do not try to treat these side effects yourself. If you have these side effects during treatment or after it ends, always contact the hospital straight away on the 24-hour number.

Your patient alert card has information about your atezolizumab treatment. Remember to keep the card with you during treatment and after treatment ends. You should show it to any doctor or healthcare professional caring for you.

Diarrhoea or tummy pain

You may have [diarrhoea](#) or tummy (abdominal) pain. Sometimes it can cause serious inflammation of the bowel called colitis.

Contact the hospital straight away on the 24-hour number if you have any of the following symptoms during treatment, or after treatment ends:

- passing more stools (poo) than is normal for you
- passing stools at night
- if you have a stoma, it is more active or needs emptied more often than usual
- watery or loose stools
- uncomfortable tummy cramps or pain
- a temperature over 37.5°C
- blood or mucus (slime) in your stool.

Effects on the liver

This treatment may affect how your liver works. It can cause liver inflammation called hepatitis. You will have regular blood tests to check how your liver is working. Tell your doctor, nurse or pharmacist if you have loss of appetite.

Sometimes liver changes can be serious. Contact the hospital straight away on the 24-hour number if you have any of these symptoms during treatment or after it ends:

- yellow skin or eyes
- feeling very sleepy
- dark urine (pee)
- unexplained bleeding or bruising
- pain in the right side of your tummy (abdomen).

If you have black or brown skin, changes in your skin colour may be less noticeable. You might first notice the whites of your eyes look yellow, or that the colour of your urine has changed.

Effects on the pancreas

This treatment can affect how your pancreas works. You will have regular blood tests to check how your pancreas is working. Your doctor may give you steroids or other treatments if tests show changes.

Contact the hospital straight away on the 24-hour number if you have these symptoms during treatment or after it finishes:

- a sharp pain in your upper tummy area (abdomen)
-

sickness and vomiting.

Skin changes

This treatment may cause inflammation in the skin. This is usually mild, but if it is not treated it can become serious. You may need creams, steroids or other medicines to treat skin problems.

Contact the hospital on the 24-hour number as soon as possible, if you notice any skin changes during treatment, or after treatment ends. This includes:

- a rash or bumps on the skin
- dry, tight or itchy skin
- areas that are a different colour to your usual skin tone.

Here are some things you can do to take care of your skin during treatment:

- Wash with lukewarm water and mild, unperfumed, soap-free cleansers.
- Moisturise your skin regularly with unperfumed moisturisers for dry skin conditions.
- Use sun cream of at least SPF 30 to protect your skin from the sun. Cover up with clothing and a hat.

Contact the hospital straight away on the 24-hour number if you have any of these serious skin symptoms:

- a rash that is spreading
- swollen, blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain.

Effects on the lungs

This treatment can cause inflammation of the lungs. This is called pneumonitis. Contact the hospital straight away on the 24-hour number if you notice any of these changes during treatment or after it ends:

- breathlessness
- a cough that does not go away
- wheezing
- a fever, with a temperature over 37.5°C.

You should also tell them if any existing breathing problems get worse. You may have:

- tests to check your lungs
- treatment, if needed.

Feeling tired (fatigue)

[Feeling tired](#) is a common side effect of this treatment. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

Sometimes, this treatment can affect glands that make hormones. This can make you feel tired. Contact the hospital on the 24-hour number if either of these things happen during treatment or after it ends:

- your tiredness gets worse or stops you doing your usual daily activities
- you feel weak or shaky.

If you feel sleepy, do not drive or use machinery.

Muscle and joint pain

This treatment can cause sore or weak muscles. Your joints may also be stiff, sore or swollen. This is called arthritis. If you already have problems with joint pain, this may become worse.

Contact the hospital on the 24-hour number if you have any of these symptoms during treatment or after it ends:

- new or worsening aches or pains
- new or worsening stiffness in a joint
- swollen joints or muscle weakness.

Effects on the bladder

This treatment can affect the bladder and cause urine infection or urinary tract infection (UTI) symptoms. Contact the hospital on the 24-hour number if you have any of these symptoms during treatment or after it ends:

- passing urine (peeing) more often
- a burning feeling when peeing
- blood in your urine
- rushing to the toilet to pee
- pain in your tummy or back
- feeling shivery
- a raised temperature.

Hormone changes

This treatment may affect your hormones. Hormones control many different processes in the body. You will have regular blood tests to check some of your hormone levels, such as your thyroid hormones.

Contact the hospital on the 24-hour number if you have any of these side effects during treatment or after it ends:

- increased sweating
- weight gain or weight loss
- dizziness or fainting
- feeling more hungry or thirsty than usual

- loss of sex drive
- passing urine (peeing) more often than usual
- headaches that do not go away
- feeling more tired than usual (fatigue)
- mood changes.

You may need drugs to control your hormone levels. For example, some people may need drugs to make the thyroid gland more or less active.

Effects on the nervous system

This treatment can affect the nervous system. This is not common. But if it is not treated, it can become serious.

Contact your doctor straight away if you have any of these symptoms during treatment or after it finishes:

- confusion
- extreme drowsiness
- headaches
- vomiting
- neck stiffness
- sensitivity to light
- dizziness or unsteadiness
- any weakness, numbness or tingling in your arms, legs or face
- fits (seizures).

You may need steroids and other treatments.

Effects on the heart

This treatment can affect [how the heart works](#). You may have [tests](#) to check how well your heart is working. These may be done before, during and after treatment.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat (palpitations)
- swollen feet and ankles.
- flu-like symptoms, such as body aches, pains and tiredness.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call **999** if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Effects on the kidneys

This treatment can affect how the kidneys work. It may cause kidney inflammation called nephritis. You will have regular blood tests to check how your kidneys are working.

Contact the hospital straight away on the 24-hour number during treatment, or after it ends, if you:

- have blood in your urine (pee)
- are passing less urine or peeing less often than usual.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an [infection](#). A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) often, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

Your white blood cell count will usually return to normal before your next treatment. You will have a blood test before having more treatment. If your white blood cell count is low, your doctor may delay your treatment for a short time, until your cell count increases.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

If you feel sick, take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Loss of appetite

This treatment can affect your appetite. Don't worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Effects on the kidneys

Bevacizumab can sometimes affect the kidneys. You will have urine tests and blood tests to check how well your kidneys are working. If tests show a lot of protein in your urine, you may need other tests to check your kidneys.

Changes to your blood pressure

This treatment can cause high or low blood pressure. Tell your doctor or nurse if you have ever had any problems with your blood pressure. Your nurse will check it regularly during your treatment. Let them know if you feel dizzy or have headaches. Your nurse might ask you to monitor your blood

pressure at home. They may give you advice on when to contact the hospital or see your GP depending on the reading.

Numb or tingling hands or feet (peripheral neuropathy)

This treatment may affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This is called [peripheral neuropathy](#). You might find it hard to do fiddly tasks such as fastening buttons or tying shoelaces.

If you have these symptoms, always tell your doctor, nurse or pharmacist. They sometimes need to change the drug or the dose of the drug. The symptoms usually improve slowly after treatment ends. But for some people they continue and are a long-term side effect of treatment.

Tummy (abdominal) pain

You may get pain in your tummy (abdomen), or have indigestion. Your doctor, nurse or pharmacist can give you advice or treatment to help.

This treatment may cause a hole (perforation) in the bowel. This is more likely if you have also had radiotherapy to the pelvis (lower part of the tummy).

Contact the hospital on the 24-hour number straight away if the tummy pain is severe or is not getting better.

It is also very important to let them know if:

- you have bleeding from the back passage (rectum) or black stools
- you are vomiting up blood
- your vomit looks like coffee grounds.

Constipation

This treatment can cause [constipation](#). Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- [Eat high-fibre foods](#), such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Changes to your taste

Some foods may taste different or have no taste. Try different foods to find out what tastes best to you. You may also get a bitter or metallic taste in your mouth. Your doctor, nurse or pharmacist can give you advice. It might help to try:

- sucking sugar-free sour or boiled sweets
-

eating cold foods

- eating sharp-tasting fresh fruit.

Taste changes usually get better after treatment ends. We have more information about [coping with changes to taste](#).

Sore mouth and throat

This treatment may cause a [sore mouth and throat](#). You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

- a sore mouth or throat affects how much you can drink or eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

Eye problems

Your eyes may become watery. Your doctor can prescribe eye drops to help with this. Always tell your doctor, nurse or pharmacist if you notice any changes in your vision.

Runny nose

Tell your cancer team if you get a runny, sore or blocked nose. They may be able to give you treatments to help.

Slow wound healing

Wounds may take longer to heal while you are being treated with bevacizumab. If you have any wounds which are not healing or look infected, tell your doctor straight away.

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

-

chest pain

- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

- be sore
- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

You can care for your hands and feet by:

- keeping your hands and feet cool by washing in cool water
- gently moisturising your hands and feet regularly
-

- wearing gloves to protect your hands and nails when working in the house or garden
- wearing loose cotton socks and avoiding tight-fitting shoes and gloves.

Difficulty swallowing and dry mouth

This treatment can cause a dry mouth or difficulty swallowing. Tell your doctor or nurse if this is a problem for you. Food cooked in a sauce or gravy may be easier to swallow. Or you could use a blender to liquidise some of your food. Taking small sips of water regularly can help with a dry mouth.

We have more information about [managing mouth problems](#).

Fistula

Rarely, bevacizumab can cause a fistula. A [fistula](#) is an abnormal opening that connects 2 or more parts of the body. If this happens, it is more likely to be in the part of the body affected by the cancer.

Blockage in the bowel

This treatment can sometimes cause your bowel to become blocked (bowel obstruction). Contact the hospital straight away on the 24-hour number if you stop passing poo and have severe pain in your tummy.

Voice changes

You may notice some voice changes or hoarseness. Talk to your doctor, nurse or pharmacist if you are worried about this.

Other important information

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

Date reviewed

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help